

I: The first question is about familiarity with AI. How familiar are you with AI technologies in general?

P: I think I use ChatGPT every day and Copilot for Microsoft, like once a month. And that's the only things I use is AI, I think.

I: And for this, I also want to get a score from one to ten. One being low familiarity and ten being high familiarity. What do you think you are on that scale?

P: Six.

I: Has your organization also introduced any AI tools or systems?

P: Yeah, there was a message on the internal education platform about Copilot. That was the only thing about AI.

I: And what was this message about?

P: Well, no, it was like you can use it if you want.

I: Okay, so just a general message, we allow you to use Copilot and that's it. But aside from that, you didn't get any training or any more guidance than just, okay, you can use this?

P: No. A few years ago I asked someone. It's an organization in the Netherlands, so it's everywhere. And I asked someone from the head office, but she didn't know. And she was like, yeah, I will message you if I know more. But I didn't get the message. And later I emailed again. No, the organization is not, how do you say that, understanding it.

I: Like supporting?

P: Yeah, supporting. It's not supporting except of the message on the portal for the people who work there.

I: Did they specify in any way what you should be using Copilot for, or just in general, you can use it?

P: It was a message like, you can use it and don't put private information about clients in Copilot. That was the only message. I thought the woman who writes the message, don't use AI.

I: So after that message, have you introduced any AI during your work?

P: Yeah, I used Copilot a little more often. But I think I use ChatGPT more because I think ChatGPT knows more about me, so can write, yeah.

I: Yeah, like it has... You've used it more, so you think it has more knowledge of what type of responses you want, right? And what type of things do you use it for currently?

P: I use it for my treatments with clients. Once there was a client who, how do you say this, unfamiliar with friendships. So I asked ChatGPT if he wants to make a picture about friendships and relationships to make it visible for him. And yeah, it was a very clear picture. And I use it and he was like, oh, I understand. And so I use it for my treatments and sometimes for my reports. And then it's very general things I ask. Sometimes I do intelligence assessments, and then I have to write some observations, and I write the observations in, yeah. Uhm.

I: like bullet points.

P: Yeah. And then I asked ChatGPT to make it like a report or a whole observation. But then with another name, always with another name. And in an observation, there's no information about the address or the school or the family situation. So those are things that I can use ChatGPT for. For the real reports, I don't use it very often.

I: So you're saying for the real report you don't use it, but there's like different types of reports. So when you make a whole report about the family situation and information about school, that's the kind of stuff...

P: Yeah, I don't use it because it's too private.

I: But there are some reports that you might make after like a shorter session or like a shorter period, which you do use it for.

P: How do you mean?

I: So you said you don't use it for writing some reports, but you do use it for some reports. So what is the distinction between these reports?

P: Yeah, for the observation of an IQ test I can use it because, yeah.

I: So there's like a specific scenario, like for example you take a test and for that you could use the report because it doesn't include all of the background information and private information. Okay, that makes sense. So do you also disclose the use with the client?

P: Uh, no.

I: Okay.

P: No, I don't know. Well, maybe sometimes, yes. Then I say to the clients, yeah, I made this picture with AI.

I: So you've mentioned that you make images that you might use during treatment to signify something, so maybe showing friendship to someone that is unfamiliar with it. And also sometimes for some reports, but then you make sure that you leave out all of the private information. So for some reports but not for others. What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think it can be nice, because if I have an interview with parents, it can be an interview for almost two hours. And then I make notes.

I: You make notes during the interview.

P: Yeah, I make notes during the interview. It takes a lot of time to make a story of a report about that. So if you have a long interview, like it's two hours, it takes a lot of time. And then I would like to use ChatGPT but that's not possible because all the information... yeah, so I won't. I will use it if I want to, but yeah, it's forbidden because of the privacy stuff.

I: Yeah, but you could use Copilot for it, or is that also not allowed?

P: I asked about Copilot, where I can use it for, because of the message on the portal. But yeah, no good answer to that question. And they were saying, yeah, you have to be very careful and don't use it for very private information. So yeah, I am very careful. I don't use it therefore.

I: So can you maybe briefly explain your type of organization? Your organization is mostly focused on younger individuals, right? And how do these people get to you? How does treatment work? Do you do intakes? Do you do treatment in a particular way, like a meeting every week or every two weeks, or is it more sporadically?

P: Well, they go to most of the time the general practitioner, and then...

I: So they just go to a GP and at some point they will send them to you, right?

P: And then I do an intake and most of the time, how do you say that, diagnostic interviews. And then treatment and most of the time I see the clients every week, for about 50 minutes.

I: Okay, that makes sense. What about intake processes? You do have an intake and you kind of mentioned that you cannot really use it there because of all of the private information, right. Have you considered using AI for preliminary screening?

P: No. It's also not my task to do that. There's a team in another part of the Netherlands to do that for us.

I: So you do do screening as an organization, but that's done by some other people?

P: Yeah, it's called a contact team. So that's the first...

I: Like the first contact between the client and your organization.

P: Yeah, but it's not really contact because it's like they're screening the report of the GP. So it's not really contact with clients.

I: Yeah, there's like some indirect contact. So the GP writes some of the problems that this person might be having and then the first line will check and see, okay, maybe we should send

this person here or maybe they don't need our help or whatever. So what about summarizing patient information?

P: What did you say?

I: I think you kind of already mentioned that maybe you can't really do that because of the private information, right.

P: Yeah. But at my organization, they have, we have many teams. I mean, it's a team for children, but there's also a team for the adults. And in the adult team, they have a... yeah. A pilot for summarizing the treatments. So it makes the audio of the treatment and then makes a summary of that and the pilot is still going.

I: Still ongoing, yes. How do you view the role of AI in tracking patient progress over time?

P: I don't know. Yeah, it can be helpful. I think maybe for, yeah, send you like a message, oh you have 6 treatments or 10 treatments, so it's time to do like an evaluation. Yeah, then it can be helpful to send a message or something like that.

I: So do you currently do monitoring or tracking of how the patient is doing? How does that happen currently in your organization?

P: Yeah, most of the time the psychologist do an evaluation between six and ten treatments. And then you discuss the progress of the client with the parents and the clients and two psychologists most of the time, instead of one. And then you discuss it. Is it still, are we still on the right track? So I think to plan evaluations.

I: And so you say between 6 and 10 sessions, then there should be some evaluation. What's the general timeline? Like how many sessions do people usually have with you?

P: I think between 10 and 20.

I: So the six and ten is like a little bit in the middle. And then would you have another evaluation after that and then possibly another at the end, or is it just one in the middle and one at the end?

P: No, no, we can plan as many as we want, but most of the time the first one is between the six and 10 treatments.

I: And then if it prolongs, so let's say 20 treatments, then you could possibly do it after 12 again, and then maybe at the end. And that's just a conversation with you and other psychologists, the parents and the client.

P: Yeah, and then you just discuss the current problems and how it's going. And at the beginning of the treatment you will make goals with the clients and you will discuss the goals.

I: So how would you feel about AI taking on monitoring tasks in your practice? You said it could send a reminder to do an evaluation. Any other thoughts on that?

P: Yeah, I think it can do many more of it. I don't know what It can do.

I: What concerns, if any, would you have about AI tracking patient progress?

P: Especially in progress, I think you have to use your own mind and your own thoughts. And yeah, if you trust AI too much, I think you don't see the clients, how he or she really is.

I: So you really think that using AI there is some risk that you stop using your own judgment and you lose track of the person that you're actually talking to?

P: Yeah.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Yeah, I think it would be nice if AI can do that for me.

I: So would you trust AI generated reports or would you need to review it?

P: Always review it. In everything AI makes.

I: Could you give an estimation on how much time it could save you if it could be doing all of those things? So generating the drafts or reports and assisting with scheduling and reminders?

P: Like between a half an hour and an hour, I think.

I: What concerns would you have about accuracy or liability?

P: Maybe AI will go too fast to conclusions, I think. Or yeah, I don't see the clients, who he or she is.

I: So it might lack a little bit of context, or it might jump to conclusions a bit fast. Would this change how you document your work if AI could support generating drafts and reports?

P: Yeah, I think maybe it can be a little, yeah. Sometimes I think my reports are a little bit long and with AI I can make it shorter and maybe sometimes a little bit more comprehensive.

I: What are your thoughts on using AI for psycho education or therapeutic exercises between the sessions?

P: Now, we use it already. So we use Therapyland. And on Therapyland are videos and information, everything about ADHD or autism or depression. Very many subjects. So if I have a client with autism, I use the right Therapyland for the clients with autism. I say you can see a therapyland or use the education videos. So I think I already kind of use that, but it's not really AI, but it's like e-health.

I: Yeah, so it's not actually an AI system, but it's just an online e-learning program that you're using.

P: Yeah, but I think it can be helpful for a client if he sees a video and asks questions to AI, like an AI bot about the video.

I: Okay, a follow-up question. What if the AI provides information that contradicts your treatment approach?

P: I don't know. I think we at our organization we use always things and information that comes from science, how do you say that. So I think AI will say the same, I hope. But I don't know if that is possible, but yeah.

I: I don't want to steer you in any way, because it's just an open question. So let's just assume that the AI is giving some other solution or some other opinion than you currently are. And even though maybe science has some judgment, maybe science is not always clear on which way to go, right. So how would you deal with a scenario where it might contradict your treatment approach?

P: I think AI has also good solutions most of the time. So then I think I will discuss it with the client. So AI says this, I say that. What do you like the most? What do you think is more working for you or helpful for you? But when it's really an advice that makes no sense, then I will discuss that with clients because then it makes no sense. But like I said, I don't think AI will give advice that makes no sense because of the science behind it.

I: So to summarize, you're open to what AI is suggesting. If it doesn't make sense at all, then you would try to explain that to your client, like hey, we don't think that this is a good idea, so we will not use this. But if it is maybe something that might benefit the client, you could discuss it and see what they would prefer, because there is also a personal factor in this, right. Like some people might respond better to other things than other people. Would you worry about clients relying too much on AI between sessions?

P: Not AI I think, but more TikTok. I don't ask clients if they ask AI or ChatGPT or if they talk to them. Because I know clients will do that. They talk to AI like it's friends or really a real psychologist. But I don't ask that to them. Most of the time I ask things about TikTok or Insta or Snapchat. What do they see there? And I warn them about risks of TikTok or Insta or the misinformation on those platforms. But I don't ask, maybe I ask too little about AI use.

I: So you do ask about their social media use in general, but not specifically on AI use. Is that mainly focused on what they see in terms of information about mental health, or also just in general their social media consumption?

P: Yeah, both. And most of the time clients call and they say, yeah, I think I have ADHD. Then I ask, why? And then they say, yeah, I saw a video about it on TikTok. And I think, yeah, I have ADHD. So then I have a conversation about it. Yeah, when they have a depression or something like that, I always ask, well, what do you see on TikTok? Because of eating disorders

or that kind of stuff. I try to ask questions about the social media news. So what they see and how many times they are on social media in a day.

I: Yeah, so you really see that social media is a big source of their information in general. How would you monitor what the AI is telling your clients?

P: Good question. I don't know. Ask them. I think I asked not enough questions about AI use. And I don't really know if they use it, but I don't ask it either, so I don't know.

I: What risks or concerns do you have about using AI in clinical work?

P: That all the information about the client is on the internet. And I think when everyone is allowed to use AI, AI may make things up about other psychologists' information. Maybe that is a risk, I think.

I: So you mentioned patient privacy and data security. Have you thought about issues like bias in AI systems?

P: I'm sorry, what?

I: Have you thought about issues like bias in AI systems?

P: What do you mean by bias?

I: So, for example, people in general might have a bias or some idea that is not necessarily true. So, for example, I might think that someone that's tall can also run fast or whatever. But that might not be the case, right. So do you think AI could also have bias? Maybe if you give some information, it would say something generalizing because it thinks, oh, this person is so and so, that's why the risk of this is so and so.

P: On one side, I think that may be the risk. But on the other side, I think AI has some kind of overview, so may know things more, the relations between problems, or the prevalence of some problems. So on the one hand I think yeah, maybe there is, but on the other hand it may also be useful.

I: Are there any other limitations that you want to talk about?

P: No, I think I mentioned everything about limitations.

I: How do you think clients respond to AI being used in their treatment?

P: I know from the pilot from the other team with adults, clients don't mind. The pilot is only for summarizing a treatment afterwards.

I: Could you also score this? So one being clients would not be open to using AI in their treatment and ten being they are very open to it.

P: I think for the children I see, maybe seven, but when the parents are there also, it may be five.

I: Makes sense. How might this affect the therapeutic alliance? Do you know the therapeutic alliance? It's just like the alliance you have between client and psychologist.

P: And what was the question again?

I: How might this affect the therapeutic alliance? So if you're introducing AI into your treatment, how might that affect that alliance?

P: I think the clients don't really, yeah, they see me. And they are still going to the location, they are still talking to a real person, so I don't know if it will affect. Maybe they're a little bit hesitant or holding back. Yeah, when they know they're making an audio of them, maybe. I don't know. I think they always see a real person.

I: Do you think clients would be more or less engaged with AI-supported treatment?

P: I don't know. Maybe it would be the same.

I: Which aspects of your work should remain strictly human and what would you need to feel confident about using AI safely in practice?

P: I think the most important thing of our work is the real contact between me and the clients. So the real conversations, the real questions, the questions like, yeah, how was your vacation or something like that. Like the personal information, the real contact between the psychologist and the client. And also how someone looks gives you very much information. So I think that's maybe the most important information in our work. And yeah, that will stay personal.

I: So that extra information is mostly nonverbal stuff, right?

P: Yeah.

I: So why do you think these aspects need to stay human? You just said it's this actual connection between two people, right. Do you have anything else to add?

P: In the beginning you asked how I use AI and something came into my mind. I also use AI for making a planning for a client. So if they are at school and they have problems with the homework planning or something like that, I make a format for that. Or I make a list with all kinds of original stuff you can do in your free time or your spare time, because the new generation only sees social media as a thing for spare time, and maybe sports. And that's most of the time the only things they like to do in their free time. And then I make a list, I ask AI, can you make a list of 100 or 150 kinds of things someone can do. So I use AI for, how do you say this, assessment for my treatments or understanding for my treatment support.

I: So you also use AI to bring up ideas to help a specific client with, okay, this client doesn't know what they should do in their free time, or maybe they should try and lower their social media or whatever. And then you ask AI, hey, come up with a hundred ideas of what I could do on a normal day. And then you might look at those and propose those to your client. Okay, that's good.